



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ايمان عبدالفتاح يونس شيمي

المحافظة : القاهرة الحي / المدينة : البساتين العنوان : البساتين

رقم البطاقة : 27906020102447 رقم التليفون : 01121260701 تاريخ تسجيل الإستمارة : 21-09-2026

العمر (تاريخ الميلاد) : 02-06-1979 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: إيمان عبدالفتاح يونس شيمي National ID: 27906020102447	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



إيمان
عبد الفتاح يونس شيمى
٣ من الجمهورية الجديدة - احمد زكى
البساتين - القاهرة

٢٧٩/٠٦/٢٠١٠٢٤٤٧

٢٧٩/٠٦/٢٠١٠٢٤٤٧

KL5436975



٢٠٢٤/٠٢ ٢٧٩/٠٦/٢٠١٠٢٤٤٧

ربة منزل

مصلحة

أنشئ



البطاقة مارية حتى ٢٠٣١/٠٢/٠٥



MDT FORM

4/12/2025 G

Name	ابراهيم عبد الفتاح يوسف	MRN	83275011
Age	46	ID	1310-2025
Menopausal status	premenopausal	Phone	011 21260701
NRS SCORE	Since 2023	Marital status	divorced
BMI	average	Family history	the mother + aunt
CO morbidity	none	REASON OF MDT	Surgery

Clinical Data

BX:	IDC
ER:	318
PR:	318
HER2:	0
Ki67:	98%

R

L

Sonomammography:

- Rt breast mass 4-7 o'clock 4.1x3.1cm
- 4x3 cm - rt axilla 1.1x0.5

Mets Workup:

CTCAP → Free
Bone scan → Suspected → then free
received 4 AC Then 4 TXT

TNM
4b1.0

Recommendations:

MDT for surgery "prior decision was MDT"
BSM → Requested & still pending

Last cycle 16/11

By extension good response of the breast
for SM+SLNB, no local relapse

18/11/2025

MDT OUTCOME & PLAN

for surgical assessment in clinic
then (SM) vs PCS + SLNB

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

د. زين العابدين
د. هادي
د. هادي

Patient Name : الأستاذة / ايمان عبد الفتاح يونس
Patient No. : 27906020102447
Path No. : 1378/25
Sex : Female
Age : 46 Y
Referring Doc :
Request No. : 13007730
Request Date : 17/04/2025
Verification Date : 05/05/2025
Financial Category : اورام دار السلام - مبادره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 16/04/2025.

Gross Examination:

- Three grayish white tissue cores up to 1.5 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+2) Grade III.
- * Ductal carcinoma in situ: Minimal DCIS.
- * Perineural invasion: Not detected.
- * Angioinvasion: Not detected.
- * Microcalcifications: Not detected.
- * Additional pathologic findings: Focal necrosis.
- * IPT (Positive control showed appropriate reactivity):
 - ER: Weak nuclear reaction in 5% of tumor cells (score 3/8).
 - PR: Weak nuclear reaction in 3% of tumor cells (score 3/8).
 - Her2: No membrane reaction of tumor cells (score 0).
 - Ki67: 98% nuclear reaction of tumor cells.

Diagnosis:

Right breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade III, minimal DCIS, and focal necrosis.

IPT:

- ER: Positive (score 3/8).

Verified by :

Dr.Toka Medhat

برجاء فتحهلف

د. انور

17

Printing Date : 06/05/2025 10:52:41

Page 1 of 3

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

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Pathology Report

PR: Positive (score 3/8).
Her2: Negative (score 0).
Ki67: Positive in 98% of tumor cells.

The cancer in this sample has a low level (1-10%) of ER expression by IHC. There are limited data on the overall benefit of endocrine therapies for patients with these results, but they currently suggest possible benefit, so patients are considered eligible for endocrine treatment. There are data that suggest invasive cancers with these results are heterogenous in both behaviour and biology and often have gene expression profiles more similar to ER-negative cancers.

Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 56% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ghonaim
Dr. Yasmine Essam

Verified by :
Dr.Toka Medhat

Printing Date : 06/05/2025 10:52:41

Page 2 of 3

Printing By : marwa mohamed youss



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Eman Abdel Fattah Yonis Sheemy	Date : 13/04/2025
Exam : MAMMOGRAPHY BOTH	Age : 46 Year(s)
Medical ID: 27906020102447	Pat. ID: 98912

Combined bilateral breast mammography and complementary ultrasound

Indication:

- Right breast 2 palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows 2 LIQ and LOQ Irregular dense mass lesion.

Complementary ultrasound revealed:

- Right Irregular hypoechoic solid masses seen at 4 and 7 o'clock with intercommunicating altered parenchyma (area about 15mm), they measured about 41x31mm and 40x30mm respectively
- Normal skin thickness, and nipple-areola complex of the right breast.
- Pathological single right axillary lymph nodes are seen with lost hila, measuring 19x11mm
- Nonspecific left axillary LN.

Opinion:

- Right breast multifocal malignant looking mass lesion (BIRADS 5) for histopathological correlation.
- Pathological single right axillary LNs.
- Benign findings left breast (BIRADS 2)

Yours truly

Signature Dr. Walaa Abdelazez

13/04/2025 2:00 PM



Patient Name: Eman Abdel Fattah Yonis Sheemy	Date : 20/04/2025
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 46 Year(s)
Medical ID:	Pat. ID: 98912

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of breast cancer.

I Chest:

- Two right breast irregular enhancing mass lesions the largest is measuring 4 X 3 cm at LIQ reaching the skin while the other is retroareolar reaching the nipple.
- Enlarged right axillary level I lymph nodes reaching 2 X 1.5 cm.
- Right lower lung lobe 3 mm nodule mostly non specific.
- Otherwise, no sizable pulmonary nodules, masses, consolidation or cavitory lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- Enlarged liver showing homogenous parenchymal attenuation. No sizable focal lesions.
- Clips of previous cholecystectomy.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys, suprarenal glands, aorta and IVC.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- Metallic structure within the right iliac fossa.
- Bulky RVF uterus.
- Left adnexal small cystic lesion measuring 2 cm.



Patient Name: Eman Abdel Fattah Yonis Sheemy	Date: 20/04/2025
Exam: C.T. ABDOMEN & PELVIS WITH CONTRAST	Age: 46 Year(s)
Medical ID:	Pat. ID: 98912

- D8 sclerotic and left iliac wing lytic tiny lesions the largest is the former measuring 6 mm.

Opinion:

Reported case of breast cancer revealed:

- No sizable pulmonary or hepatic metastatic deposits.
- Few osseous lesions for bone scan.

Dr. Mohamed Mohsen, M.D.

Patient's Name:
Date:
Referred by:
Hospital:

Eman Abdelfatah Younes
2026-05-02
Prof.Dr. CAPwithC
Goustave Roussey International Egypt

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MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA underwent right MRM. No available previous films for comparison.

I- CHEST:

- Mild emphysematous changes of both lungs evidenced by bilateral scattered areas of air trapping.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ **No pulmonary or hepatic metastatic lesions.**

Dr.sherif Wagdy

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